



2025 EARLY CHILDHOOD LANDSCAPE SURVEY REPORT



UNITED WAY
Greater Austin

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Executive Summary

High-quality early care and education (ECE) is foundational to children's healthy development, families' economic stability, and the strength of the local community. Yet the ECE system operates under persistent strain, shaped by workforce shortages, financial fragility, and uneven access to affordable care. This 2025 Austin/Travis County ECE landscape survey offers a timely snapshot of local conditions during a critical transition period: after the end of the American Rescue Plan Act (ARPA) funding but before the disbursement of Raising Travis County funds. It was conducted between late October and December of 2025, and it provides a baseline understanding of the current conditions and challenges facing our childcare providers and is intended to strengthen coordination, accountability, advocacy, and funding decisions in the community.

By capturing the experiences of both child care centers and home-based providers, this survey highlights the interconnected challenges that make it difficult to operate stable, high-quality programs. Providers reported difficulty keeping classrooms fully enrolled, retaining existing teachers, recruiting qualified staff, maintaining facilities, and keeping tuition affordable. On average, providers reported operating at just 65% of their licensed capacity, and 72% of desired capacity with approximately 2,562 open slots. Only 16% of providers were fully enrolled, while 84% had open slots in three or more age groups. Teacher turnover exceeded 40%, driven by low wages, career changes, and performance-related dismissals. Well over half of the providers indicated that open positions remained unfilled largely because applicant pools were small and candidates lacked required qualifications. These staffing challenges often led to operational shifts, such as additional workload for current teachers, closing or combining classrooms, or inability to meet quality standards, further straining an already fragile system. At the same time, providers were dealing with rising costs for labor and benefits, food, classroom materials, and facility maintenance. About a third of providers identified the inability to pay competitive wages and benefits as major hiring barriers. Facility upgrades ranked among both the top concerns, and top funding priorities. Over one-fifth of providers were not confident in their financial stability over the next year.

These findings help illuminate where the system is most vulnerable, and where targeted support can have the greatest impact. Top challenges and funding priorities for providers align, and include increasing enrollment, staff wages, providing or improving staff benefits, supporting staff education or training, and upgrading facilities.

This report is United Way's first step in sharing an initial overview of the data with providers, funders, coalition members, and the broader community. In the months ahead, United Way will build on this work through topic-specific reports (such as enrollment, quality, wages and benefits), and dashboards on the Early Childhood Data Hub. United Way will work with providers, coalition partners, funders, and community members to interpret the findings together and identify priority for deeper analysis.

Background

Since the onset of COVID-19, early care and education (ECE) providers have faced persistent challenges, including staffing shortages and financial instability. In 2025, 15 centers and 8 home-based providers closed permanently, continuing a trend of slightly higher closure levels in 2024. The successful Travis County Tax Rate Election in 2024 (now the Raising Travis County initiative) was a huge milestone for ECE in our region. However, to ensure these funds are used effectively, implementation plans must be informed by baseline data that is currently not available. Further, we must collect data on aspects of the ECE landscape beyond the scope of the Raising Travis County initiative to guide future programs, coalition priorities, and policies.

United Way for Greater Austin previously led biannual surveys of child care providers (2010–2018) to inform landscape analyses and to support coalition strategic planning. These survey efforts were paused during the pandemic and were not relaunched until the Fall of 2025, when collecting current local data on key indicators, such as enrollments and waitlists, staffing and qualifications, compensation, and pressing needs and challenges experienced by providers, became essential. To ensure high-quality, actionable data and reduce survey fatigue, the 2025 survey was co-created with key partners, including ECE providers. Providers who contributed to survey design, as well as those who completed the survey, were compensated for their time.

This updated data offers a timely snapshot of the ECE landscape during a critical transition period, after the American Rescue Plan Act (ARPA) funding has ended but before Raising Travis County funds were dispersed. It provides a baseline understanding of the current conditions and challenges facing our provider community. These insights can strengthen coordination, accountability, advocacy, and funding decisions across the ECE sector and will be available publicly through a variety of reports, white papers, and on United Way's new Early Childhood Data Hub. Moving forward, United Way hopes to conduct portions of this survey annually to support long-term planning, system improvements, and evaluation.

This initial report offers a high-level snapshot of several indicators that have been identified as priorities by providers, coalition members, and funders - including information on enrollment and capacity gaps, subsidy and quality, tuition rates, non-traditional hour care, workforce stability and challenges, teacher and director demographics and wages, as well as provider challenges and funding priorities. Additional data on topics such as program support, professional development, and child demographics, was collected but is not included in this current report. More detailed findings, open-ended responses, and topic deep-dives will be shared over the coming months through dashboards, white papers, and community discussions.

Survey Distribution Method

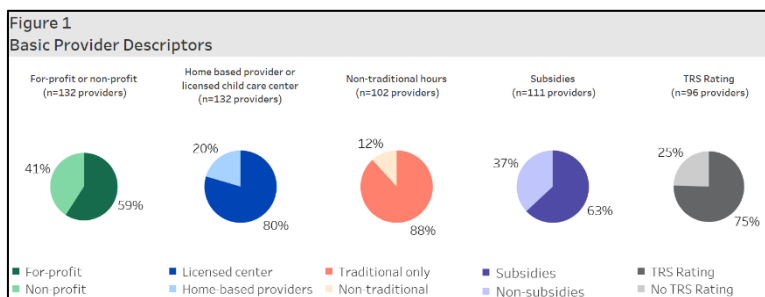
A comprehensive list of early care and education providers in Austin and Travis County was created by combining two State of Texas data sources: the Texas Health and Human Services (HHS) Child Care Regulation dataset and the HHS DC Monthly and Weekly Status Change datasets. In total, the list counted 572 open and operating licensed child care centers or registered/licensed home-based early care and education providers in Austin and Travis County. We excluded providers licensed only for before- or after-school care, providers serving exclusively school-age children, and listed home providers.

United Way emailed the survey directly to all eligible providers multiple times and partnered with Austin Public Health, Workforce Solutions, and several community organizations to broaden distribution. The survey was also shared on United Way's social media channels. Approximately 64 providers—primarily home-based— did not have email addresses listed in their HHS records. These providers were contacted up to three times by phone to ensure they received information about the survey. United Way offered technical assistance to anyone seeking additional support, including phone or in-person help, and provided multiple options for completing the survey, such as a paper version or the ability to submit spreadsheets for providers with multiple locations.

The survey remained open from late October through late December to maximize participation. Providers who began but did not complete the survey received follow-up emails and phone calls offering additional support to help them finish. Respondents were compensated for completing the survey with gift card amounts ranging from \$100 - \$200 per site/provider.

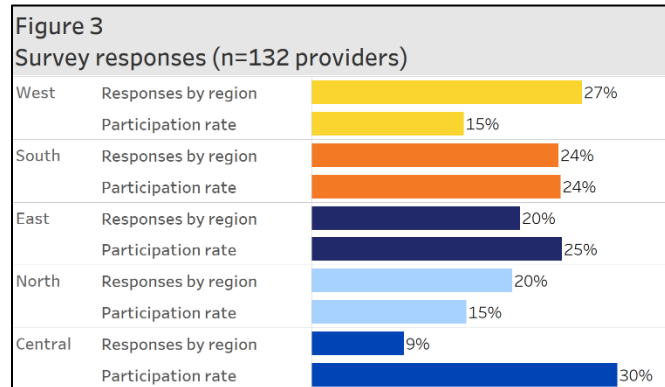
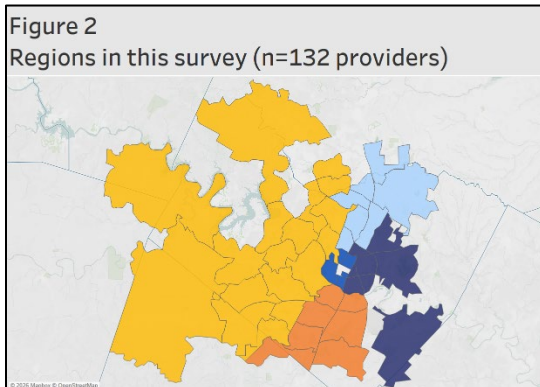
Descriptive Data

In total, 132 child care providers within Austin and Travis County started the 2025 Early Care and Education survey, with 100 providers completing it. Respondents were primarily owners or directors, and other administrative staff. 59% were for-profit providers, 80% were licensed child care centers, 88% only provided traditional care hours, and 63% enrolled children receiving child care subsidies/scholarships from the local workforce board, and 75% of providers reported having some level of Texas Rising Star (TRS) accreditation with the majority reported having TRS 4-Star accreditation (Figure 1).



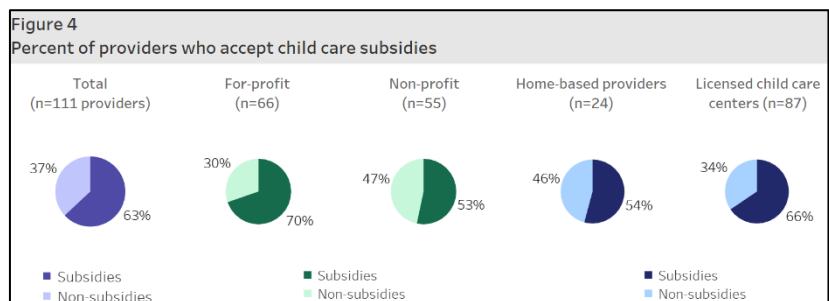
Regional Representation

When comparing responses by geographic region, most came from providers in the largest region, West Austin (27%) followed by South Austin (24%). North and East Austin each accounted for a fifth of all responses. Central Austin, the smallest region, had the fewest number of responses (9%). When accounting for the number of providers in each region, the proportion of responses varied across all five regions. Central Austin had the lowest number of providers out of all five regions; however, it had the highest participation rate of all five regions (30%). In contrast, West Austin had the most providers but was tied for the lowest participation rate alongside North Austin (15%) (Figures 2 & 3). Additional analyses are needed to assess detailed differences in regional representation, but response rates were highest among licensed centers serving children with subsidies and lower among centers not accepting subsidies and home-based providers – many of which are located in the regions with the lowest participation rates.



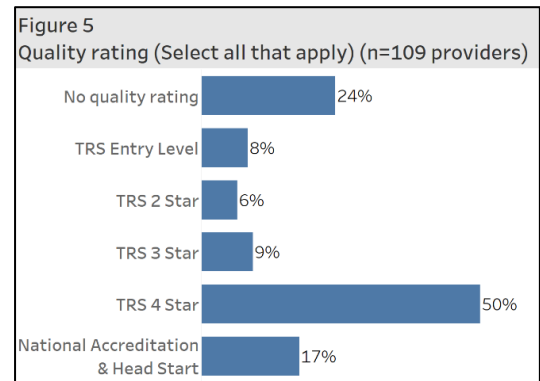
Child Care Subsidies/Scholarships

Providers varied in their participation in the Workforce Solutions child care subsidy/scholarship program (CCS). Overall, 63% reported that they accepted child care subsidies. Among those that did, the median number of enrolled children receiving child care scholarships/subsidies was 5, with the highest number of children being 85. When comparing for-profit and non-profit providers, 70% of for-profit providers and 53% of non-profit providers reported serving children with subsidies. Home-based providers and licensed child care centers also showed differences, albeit to a lesser extent, with 54% of home-based providers and 66% of licensed child care centers serving children with subsidies (Figure 4).

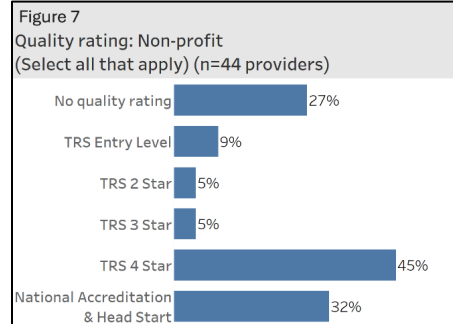
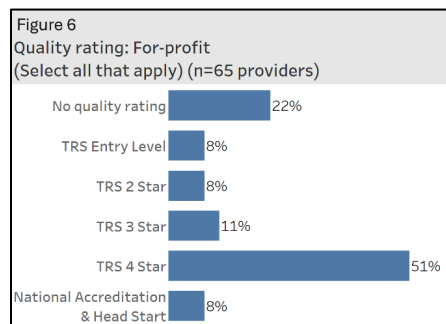


Quality Rating

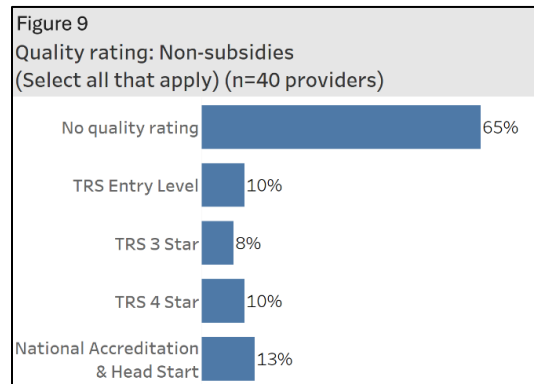
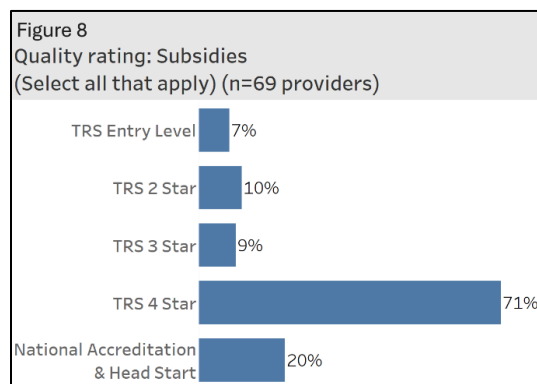
Quality ratings give families and the broader community a shared way to understand whether local child care providers are offering safe, developmentally supportive environments. Providers were asked whether they were quality-rated with an option to select multiple ratings. 73% of providers reported having some level of TRS accreditation, with the majority reported a TRS 4- Star rating. Only 24% of providers reported they did not have any quality rating (Figure 5).



For-profit and non-profit providers had similar rates of TRS accreditation, but their national and Head Start accreditation rates differed greatly: 8% of for-profit providers compared with 32% of non-profit providers (Figures 6 & 7).



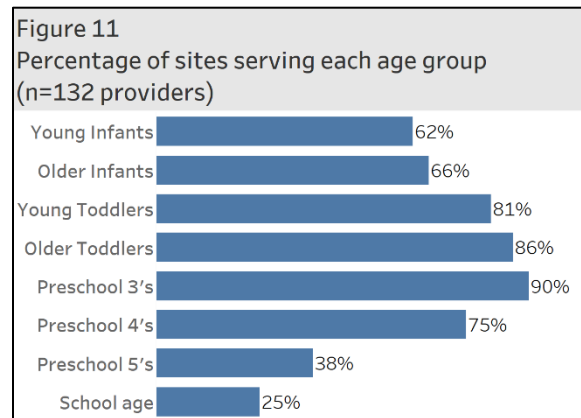
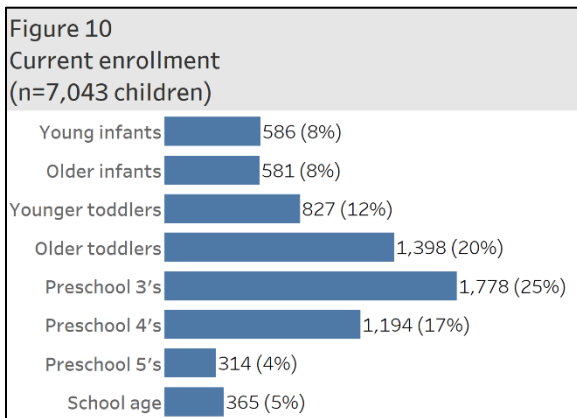
Quality ratings differed substantially between providers that accepted child care subsidies and those that did not, which is unsurprising given that TRS participation is required to serve children on subsidies. Among subsidy-accepting providers, 71% were TRS 4-Star and 20% held national accreditation or were Head Start affiliated. In comparison, 65% of providers outside of the subsidy system had no quality rating, and only 10% were TRS 4-Star (Figures 8 & 9).



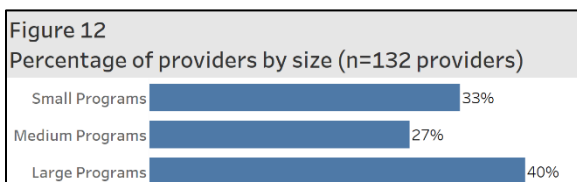
Enrollment and Capacity Gaps

Communities often rely on licensed capacity to estimate how many child care seats exist in a region because this data is easy to access and represents the maximum number of children a permit holder may care for at one time. However, licensed capacity does not reflect actual enrollment, as providers rarely operate at full capacity due to factors like staffing shortages, age-group constraints, and intentional efforts to maintain quality standards. To better understand true enrollment levels and capacity gaps, providers were asked to report their current and desired enrollment in the following age groups: Young infants (birth–11 months), older infants (12–17 months), young toddlers (18–23 months), older toddlers (24 – 35 months), preschool 3's (36 – 47 months), preschool 4's (48 – 59 months), preschool 5's (60 –71 months), and school-aged (6 years and older).

On average, providers were operating at just 65% of their licensed capacity. Across all providers, licensed capacity totaled 11,099 seats, yet only 7,043 children were reported as currently enrolled. Almost half of children served were preschoolers, a third were toddlers, and only 18% were infants. Providers most commonly served older toddlers (86%) and preschool 3's (90%). Overall enrollment patterns generally aligned with the share of programs serving each age group. The main exceptions were infants, where total enrollment was lower (Figures 10 & 11). However, this pattern is consistent with required infant to teacher ratios and group size limits, which naturally cap enrollment. The median enrollment varied by age group, ranging from six children for young infants to fifteen children for preschool 3's.



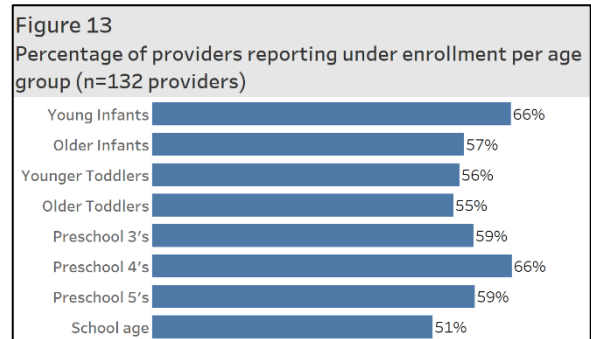
Past reports indicate that enrollment size shapes staffing patterns, tuition structures, and operational priorities. For this analysis, providers were grouped into three categories: Small (1 to 20 children), medium (21-50 children), and large (50+ children). Overall, small, and medium providers were fairly evenly represented, while large providers made up the largest share at 40% (Figure 12).



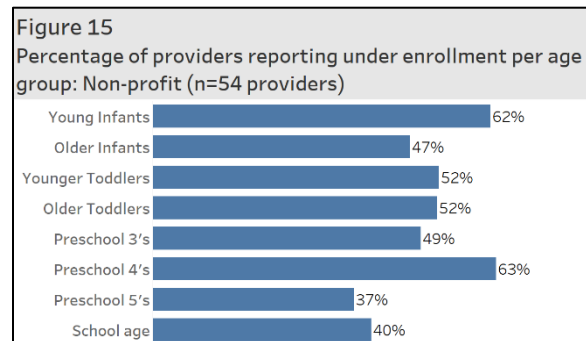
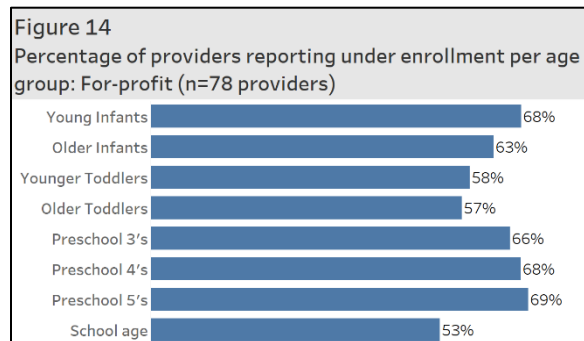
Capacity Gaps

Under-enrollment is a serious issue for providers as it directly undermines the financial stability and operational viability of their programs. While many providers do not have a desire to operate at maximum licensing capacity, it is important to understand what actual capacity gaps exist in the community.

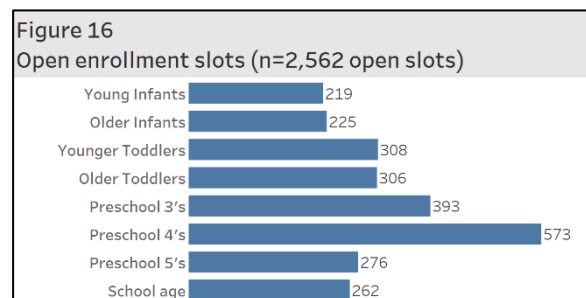
To assess true under-enrollment, the survey gathered data on both current and desired enrollment for each age group. In total, providers reported operating at 72% of desired capacity with approximately 2,562 open seats. Only 16% of providers reported themselves as fully enrolled. Overall, the majority of providers reported under-enrollment across all age groups, with the highest number of providers reporting open seats for young infants and preschool 4's (Figure 13).



Compared to for-profit providers, non-profit providers were less likely to be under-enrolled across all age groups. Differences were especially large for older infants, preschool 3's and preschool 5's (Figures 14 & 15).



When examining the number of open slots, there are the fewest available for the youngest and oldest children. Preschool 4's had the largest availability, with 573 open slots, more than twice the number reported for young infants (219 slots) (Figure 16).

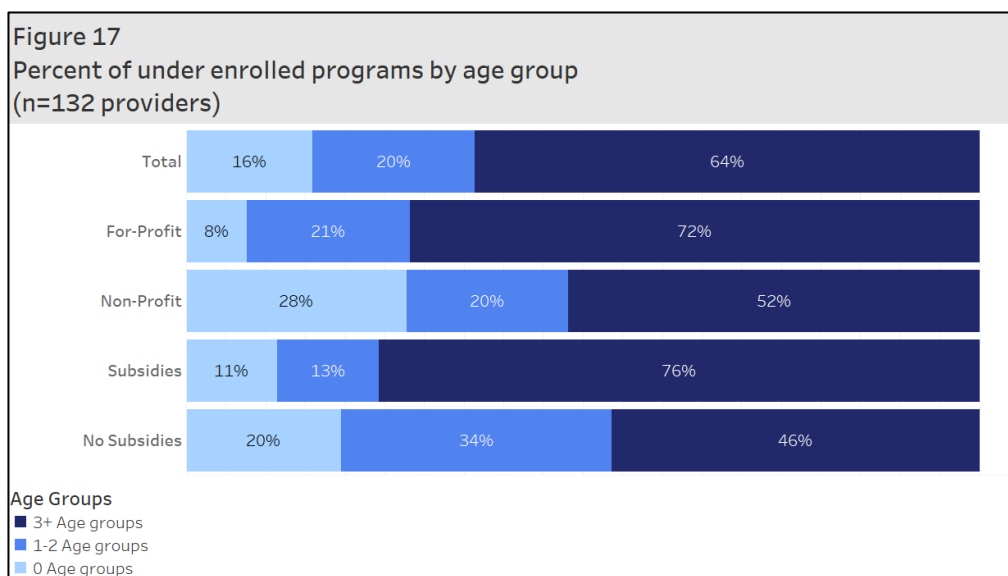


The data above does not provide information about the severity of under-enrollment. Some level of open slots is expected due to routine turnover due to children aging into new classrooms, children aging out of child care, families relocating, or changes in care schedules or family circumstances. However, current under-enrollment rates appear to be significant.

84% of providers reported having at least one age group under-enrolled and 64% reported having three or more age groups under-enrolled (Figure 17). However, despite under-enrollment being fairly high across providers, only 51% of programs reported having a waitlist.

Patterns of under-enrollment differed notably between for-profit and non-profit providers. While the percentage of providers reporting under-enrollment in one age group was similar, 72% of for-profit providers reported having under enrollment in 3+ age group compared to only 52% of non-profit providers. (Figure 17).

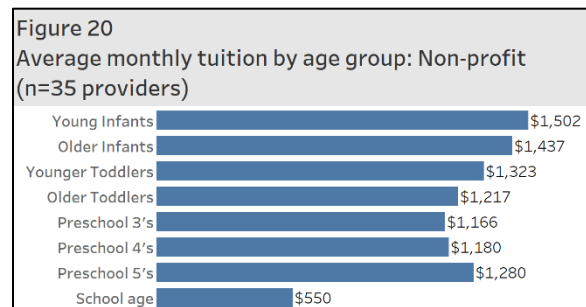
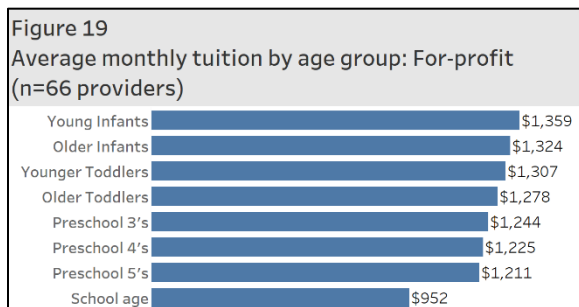
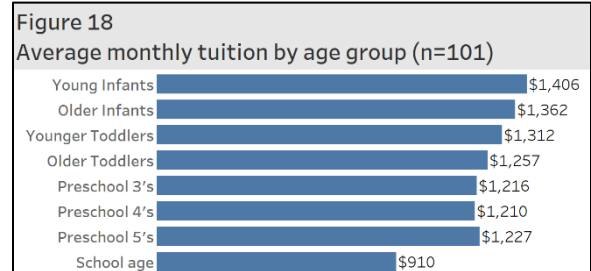
Similarly, patterns of under-enrollment also differed greatly between providers accepting child care subsidies/scholarships, with subsidy-participating providers experiencing broader enrollment challenges. Over three-quarters of the providers accepting children on CCS were under-enrolled in more than three age groups, compared to slightly less than half of providers not accepting children on CCS (Figure 17).



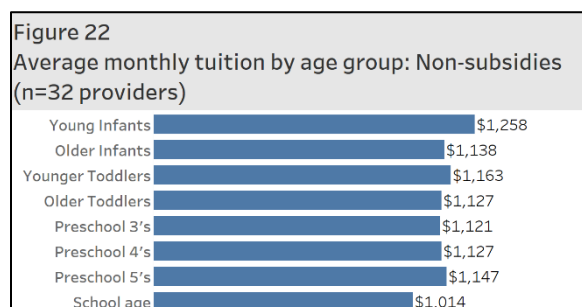
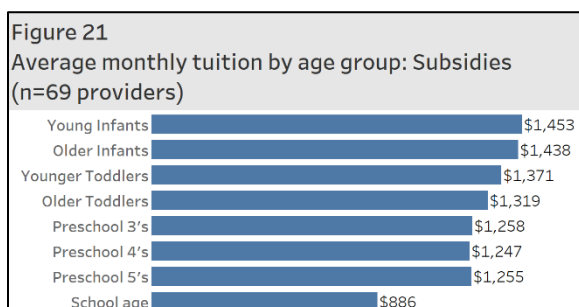
Tuition

Tuition rates matter as they reveal how accessible, sustainable, and equitable the local child care system is for families. Additionally, they affect child care subsidy reimbursement, which cannot exceed the listed tuition. The average monthly tuition rate across all ages was \$1,260. Focusing specifically on children under 6, tuition was the highest for young infants (\$1,406/month) and lowest for Preschool 4's (\$1,210/month) (Figure 18).

Average monthly tuition was higher for non-profit providers compared to for-profit providers for most age groups except preschool 3's, 4's and school age children (Figures 19 & 20).



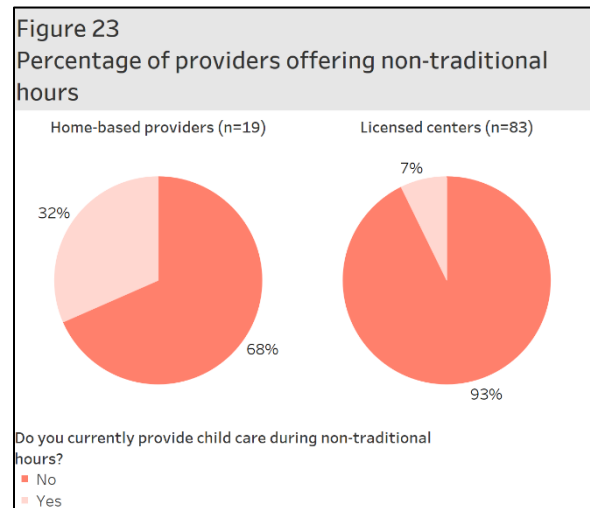
When comparing providers that do and do not accept child care subsidies/scholarships, tuition differences become more pronounced. For every age group except school-age, providers accepting subsidies reported higher tuition. The gap was especially large for infants, toddlers, and school age children (Figures 21 & 22).



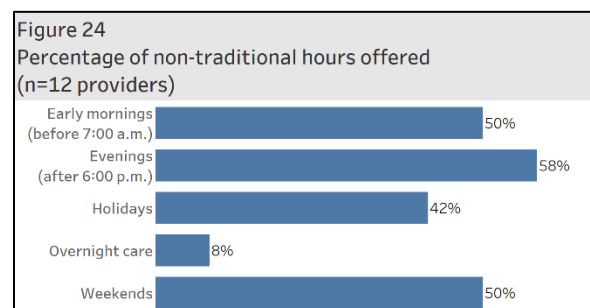
These high tuition rates may also be contributing to enrollment challenges, as \$15,000 per year, per child, may be a large barrier for many parents, especially those with infants or multiple children. In Travis County, the current median annual income for families with children is \$135,463, while the median annual income for female-headed households with children is \$36,537. While the average tuition for one child may account for only 11% of the typical family's income, it is as much as 41% for a single mother's income – and that is before taxes, rent, and other essential expenses.

Non-Traditional Hours

A large share of the workforce, such as hospital staff, service workers, and retail, transportation, manufacturing, and hospitality employees, work schedules that do not align with traditional child care hours. Looking at how many providers offer care during very early mornings, evenings, nights, and weekends helps communities understand what options are available to these families. Only 12% of all providers offered non-traditional hours. Home-based providers reported much higher rates of providing non-traditional hours as compared to licensed child care centers (Figure 23).



Among those that offered non-traditional hours, evenings (after 6:00 p.m.) were reported by more than half of the providers. Overnight care on the other hand was reported only by one provider (Figure 24).

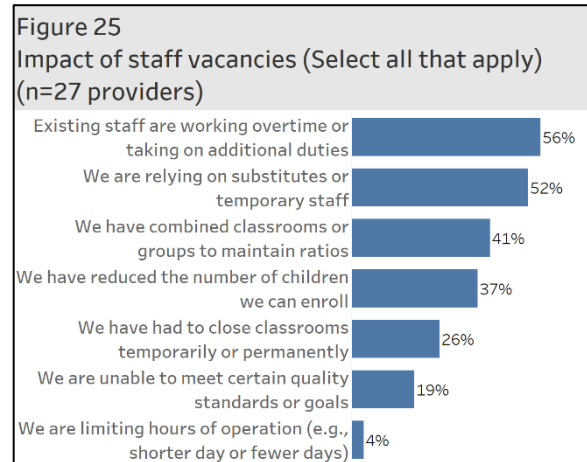


Workforce Stability and Challenges

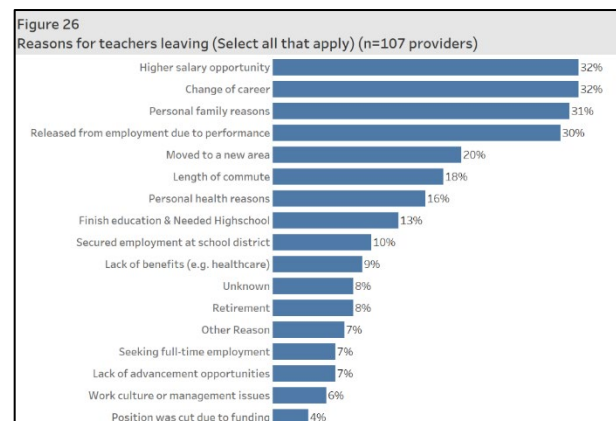
Data on staffing and teachers' education, wages, and benefits is essential information to understand whether our early care and education system has the capacity, stability, and quality needed to meet families' needs. Staffing is the backbone of the entire system, and understanding it helps explain many of the challenges communities face such as shortages, waitlists, capacity and quality gaps, as well as provider closures.

Over a quarter of providers (26%) reported being understaffed with a total of 63 open positions. There were slight differences when viewed by for-profit and non-profit providers. 22% of for-profit providers reported being understaffed compared to 31% of non-profit providers. A similar difference was reported between providers who accepted children on subsidies and those who did not (22% and 30% respectively).

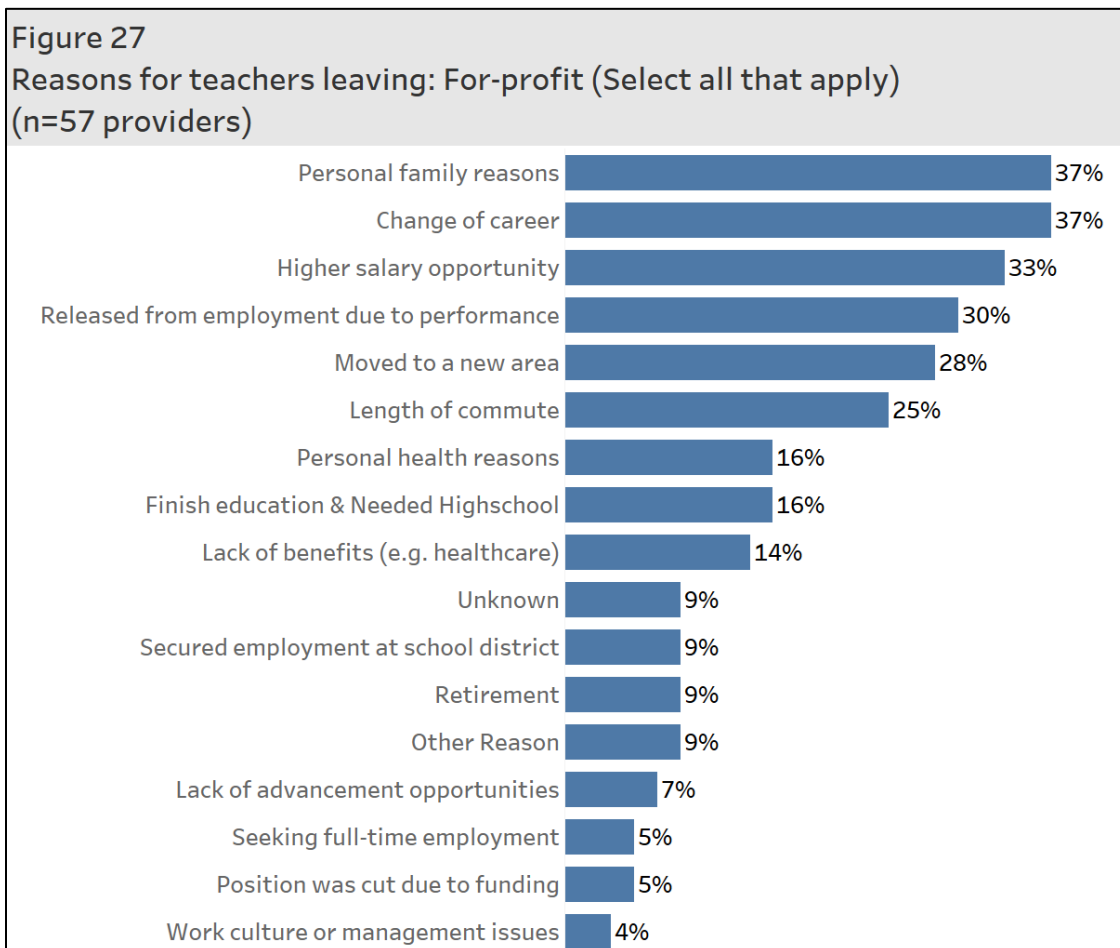
Only one provider reported that staff vacancies were not affecting operations. All others reported some level of impact. The most common consequence was existing staff working overtime or taking on additional duties (56%), while the least common was limiting hours of operation (4%). Staff vacancies also appear to significantly impact the number of children providers can serve. Over a quarter of providers closed classrooms, more than a third reduced enrollment, and two-fifths combined classrooms or groups (Figure 25).



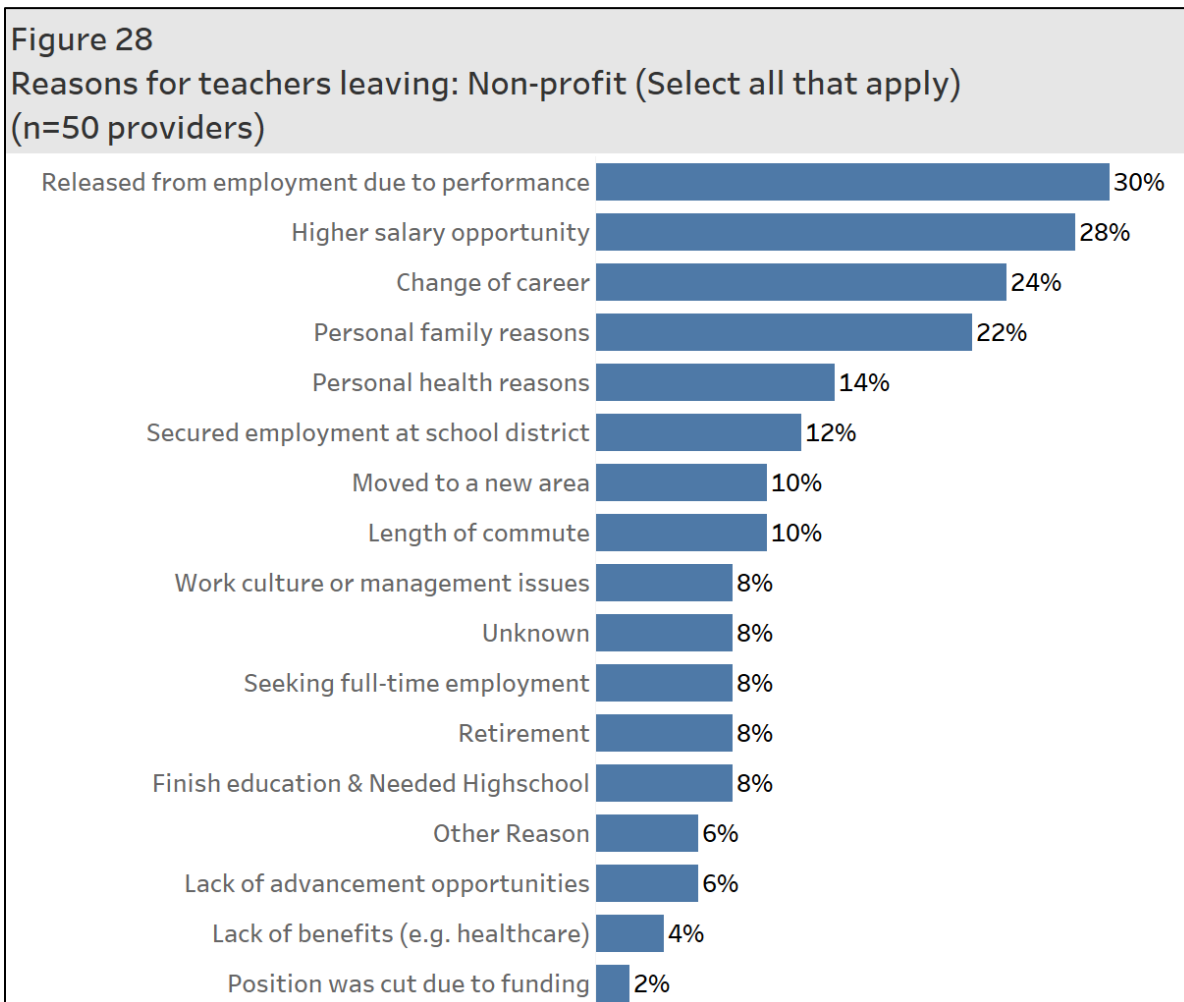
Teacher turnover appears to be a major factor contributing to understaffing and open positions. Providers reported that 418 teachers left their program within the last 12 months, comprising 46% of the reported number of teachers employed within the same time period. Respondents were asked to indicate why teachers had left their program by selecting any applicable reasons from a list of 16 options. Teachers were most likely to leave to pursue a higher salary opportunity (32%) or a change in career (32%), for personal family reasons (31%) or being released due to performance (30%) (Figure 26).



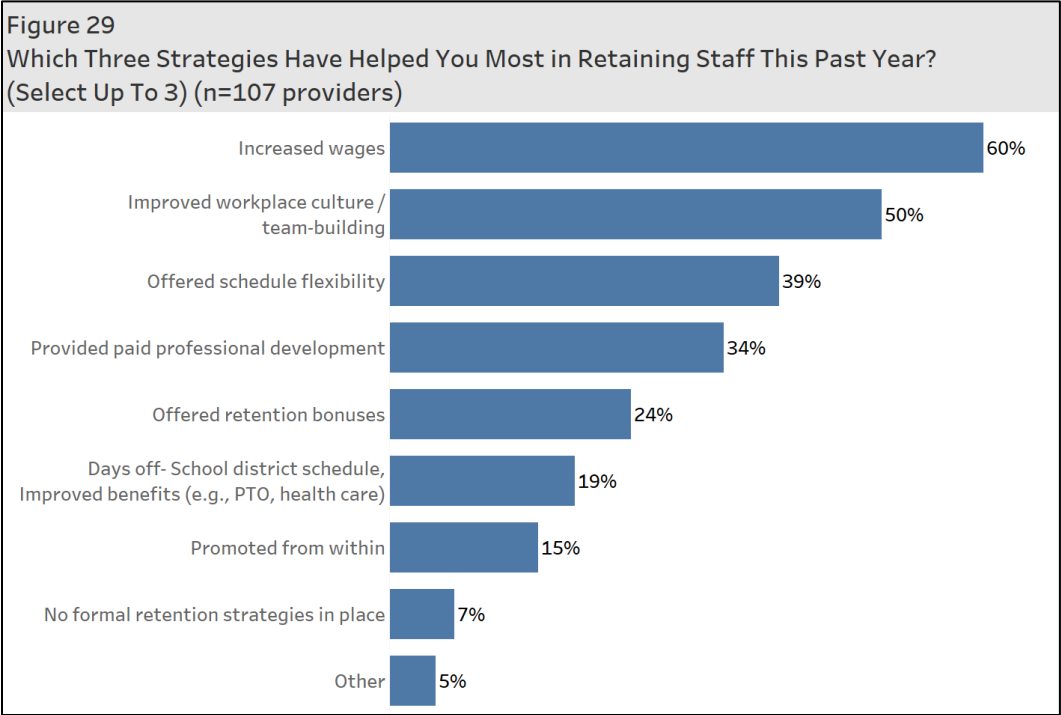
Among for-profit providers, the top four reasons for teachers leaving were the same, but the order shifted slightly. Personal family reasons and career change were tied as the most frequently reported reasons for leaving (37%), followed by higher salary opportunities (33%), and performance issues (30%) (Figure 27).



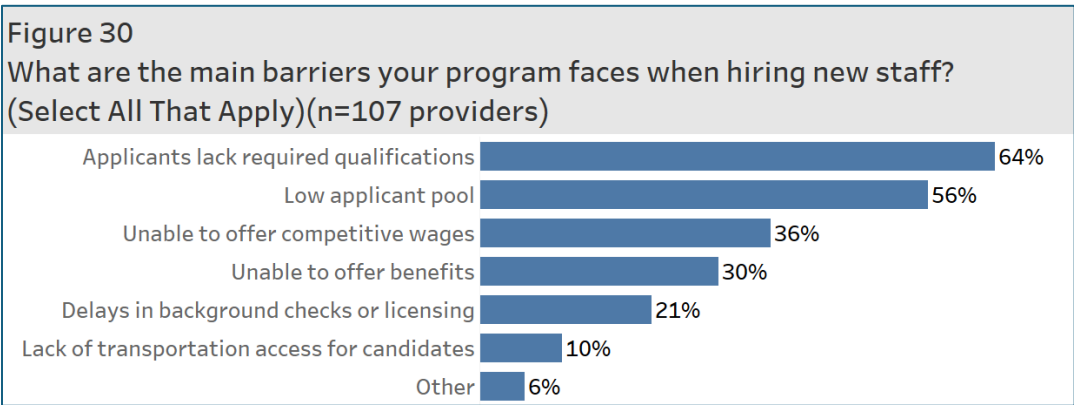
Among nonprofit providers, the top four reasons for teachers leaving were the same, but the most common reason was due to being released from employment due to performance (30%). This was, however, followed closely by teachers leaving for a higher salary opportunity (28%), change of career (24%), and personal family reasons (22%) (Figure 28).



Respondents were also offered eight teacher-retention strategies and were asked to select the three strategies that had been most effective in retaining teachers. The three highest reported retention strategies were increased wages (60%), improved workplace culture/team building (50%), and offering schedule flexibility (39%) (Figure 29).

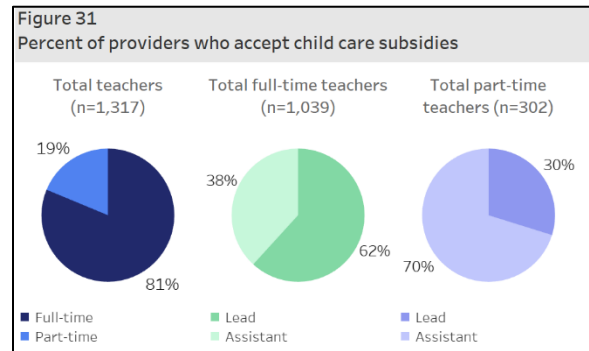


Providers were given a list of six hiring barriers and asked to select all that applied. The most frequently reported barriers were applicants lacking required qualifications (64%) and a low applicant pool (56%). These were reported at nearly twice the rate of the next most common barriers — being unable to offer competitive wages (36%) and benefits (30%) (Figure 30).

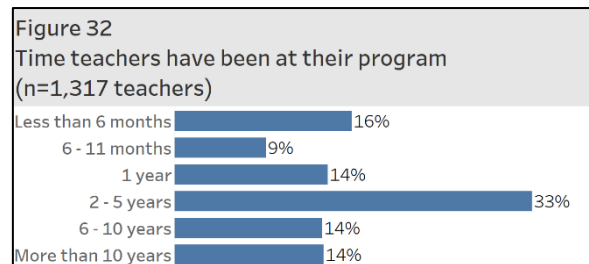


Teaching Staff Information

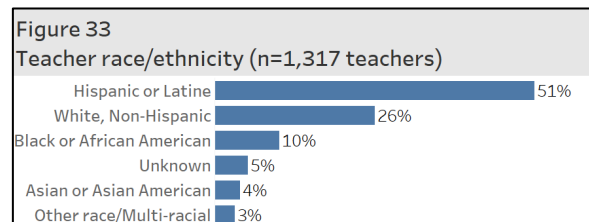
Understanding the ECE landscape requires a clear picture of the teaching workforce providing care to children and families. Providers reported a total of 1,317 teachers, 81% of whom were full-time. Among the 1,039 full-time teachers, 62% were lead teachers and of the 302 part-time teachers 30% were lead teachers. (Figure 31).



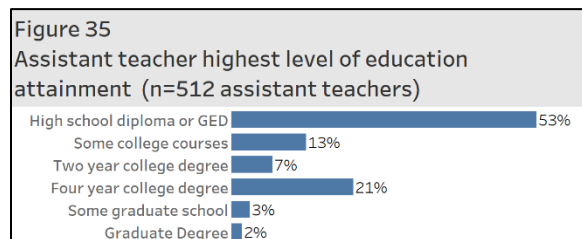
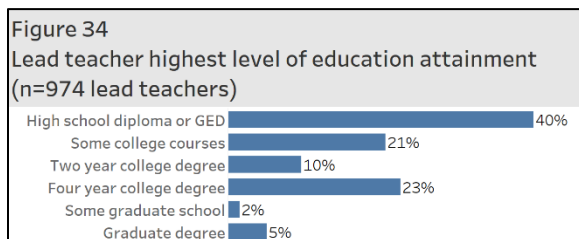
Most teachers (61%) had been working with the same provider for more than two years. A quarter of the teachers had been there less than one year and 14% had been there for more than 10 years (Figure 32). There was no notable difference in teacher tenure between for-profit and non-profit providers.



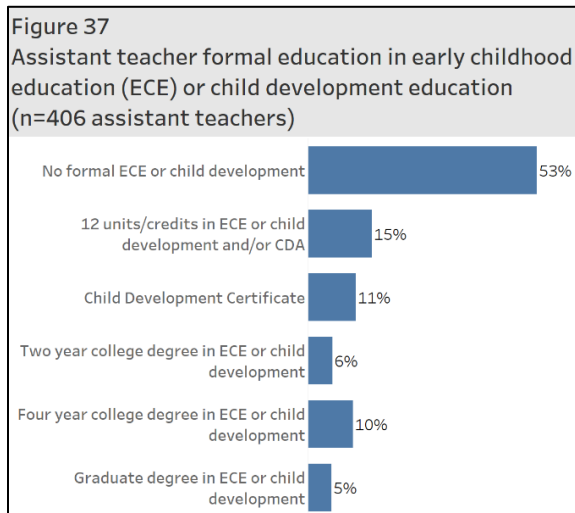
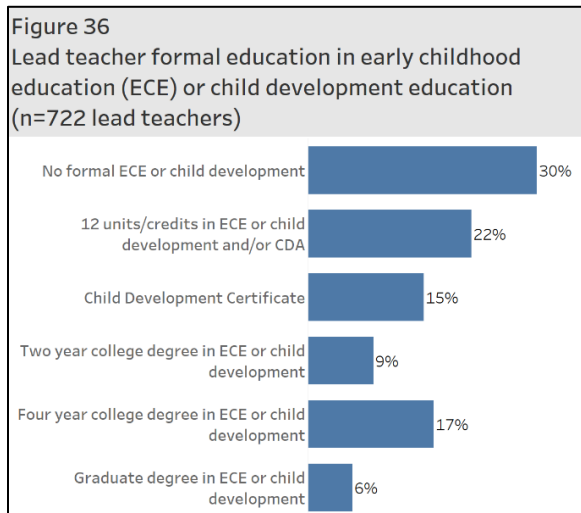
Nearly three-quarters of all teachers were teachers of color with the majority being Hispanic/Latine (51%) (Figure 33). There was no notable difference in race/ethnicity between for-profit and non-profit providers.



Teacher education is important for the care children receive, and the long-term outcomes of children. While 30% of lead teachers and 26% of assistant teachers had at least a four-year college degree, 40% of lead teachers and 53% of assistant teachers still only had a high school diploma or GED (Figures 34 & 35).

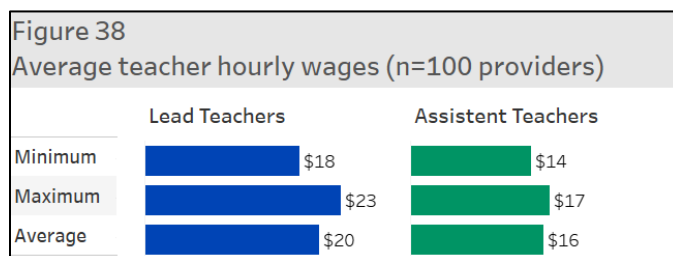


In terms of specialized child development education, lead teachers had much higher rates of formal child development education compared to assistance teachers. Almost half of lead teachers had at least 30 credit hours of formal ECE education (a requirement for high quality Texas Rising Stars accreditation) compared to only a third of assistant teachers (Figures 36 & 37).



Teacher wages shape who enters the field, who stays, the quality of care children receive, and the long-term stability of the entire child care system. While the findings on average hourly wages show improvements compared to previous years, there remains significant variability across providers. Moreover, averages hourly wages are still low relative to other fields and fall short of providing teachers with a living wage in Austin/Travis County.

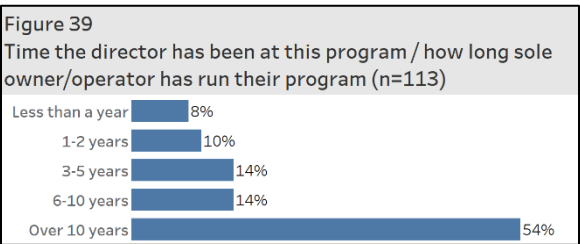
On average, providers reported paying their lead teachers at higher rates (\$20/hour) compared to assistant teachers (\$16/hour). For lead teachers, the average minimum wage was \$18/hour, with the lowest being \$10/hour and the highest being \$30/hour. The average maximum wage was \$23/hour with wages ranging from \$13-\$40/hour. In comparison, for assistant teachers the average minimum wage was \$14/hour, with the lowest being \$11/hour and the highest being \$26.5/hour. The average maximum wage was \$17/hour with wages ranging from \$12.5-\$26.5/hour (Figure 38).



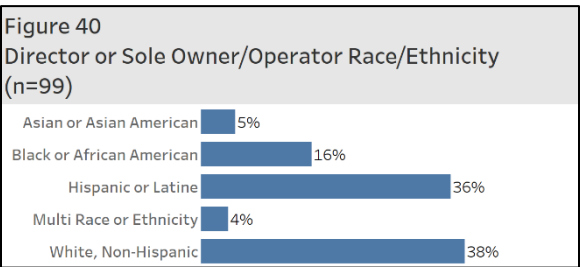
Directors and Sole Owner/Operators Information

Directors and owners are central to how early childhood programs function as they shape quality and daily practice. They set expectations for operations, curriculum, teacher-child interactions, classroom environment, family engagement, and the work environment for staff.

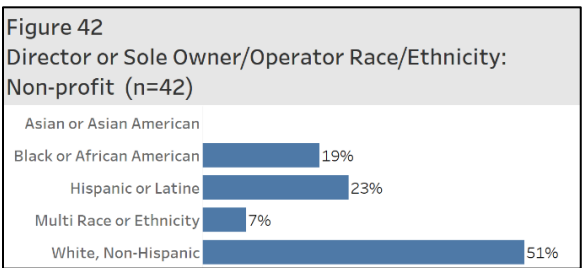
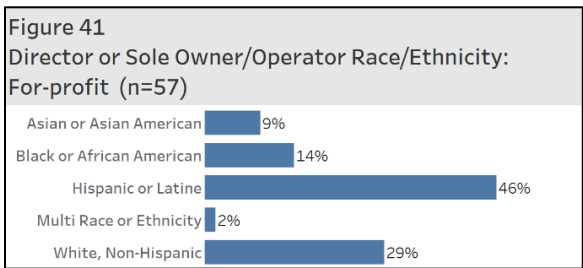
Over half of all the respondents reported that the director or owner had been at their respective program for over 10 years, and over 90% reported being at their programs for at least one year (Figure 39). There was no notable difference between for-profit and non-profit providers.



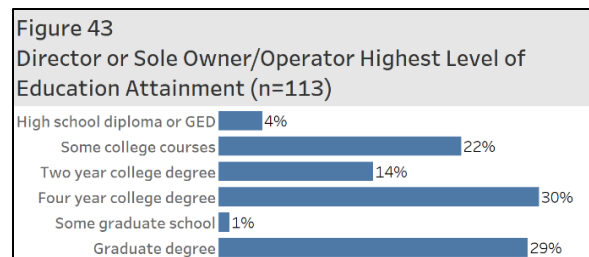
While 62% of directors were people of color, the majority were White, Non-Hispanic (38%) followed closely by Hispanic/Latine (36%) (Figure 40).



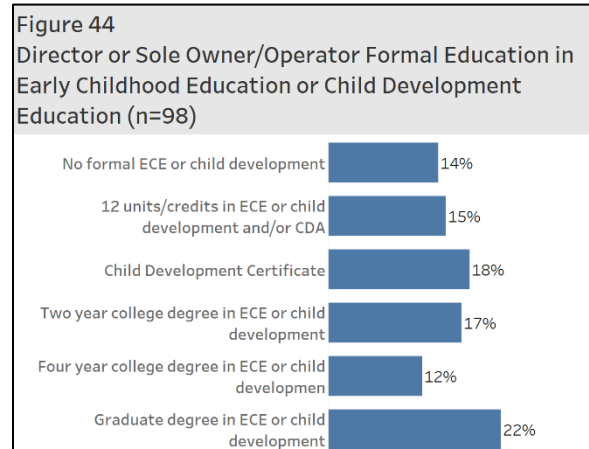
The race and ethnicity of the director or owner differed greatly by type of operation. For-profit providers were most often led by Hispanic/Latine directors or sole owner/operators (46%) followed by White, Non-Hispanic directors or sole owner/operators (28%). In contrast, non-profit providers were predominately led by White, Non-Hispanic directors or sole owner/operators (51%), with Hispanic/Latine leaders representing the second-largest group (23%) (Figures 41 & 42).



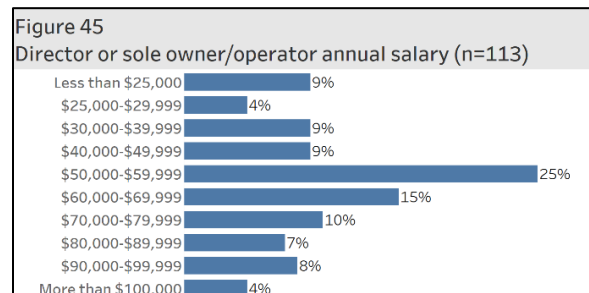
Education levels among directors varied with 60% having at least a four-year college degree, and 74% having at least a two-year college degree. Only 4% had only a high school diploma or GED (Figure 43).



In terms of specialized child development education, 14% had no formal ECE education whereas 34% had a four-year college degree or a graduate degree in ECE. The most common reported formal education level among directors or sole owner/operators was a graduate degree in ECE or child development (22%) (Figure 44).



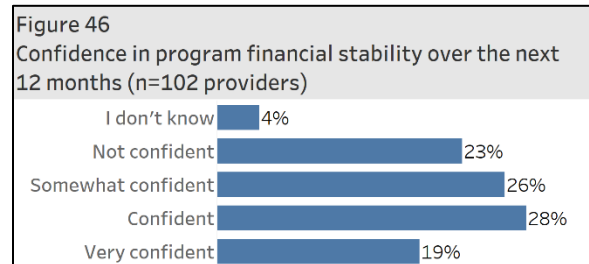
A little over half of the directors and sole operators/owners reported an annual salary of less than \$60,000 a year. Most directors and sole owner/operators reported an annual salary between \$50,000 and \$59,999 (25%) and nearly 9% reported an annual income of less than \$25,000 (Figure 45).



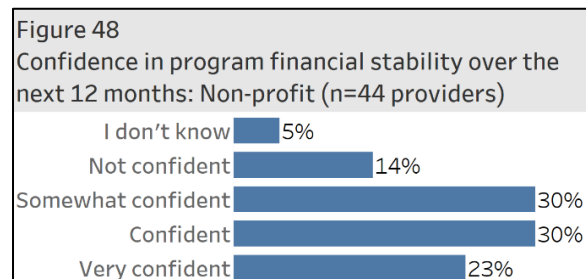
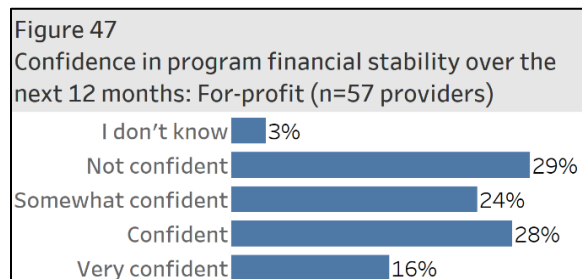
Provider Financial Confidence

To better understand the stability of the early care and education system and the supports programs may need, providers were asked how confident they felt in their program's financial stability over the next 12 months.

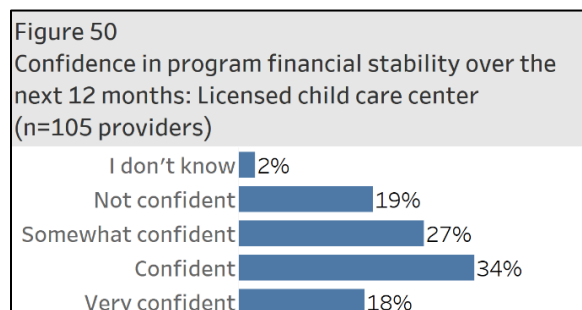
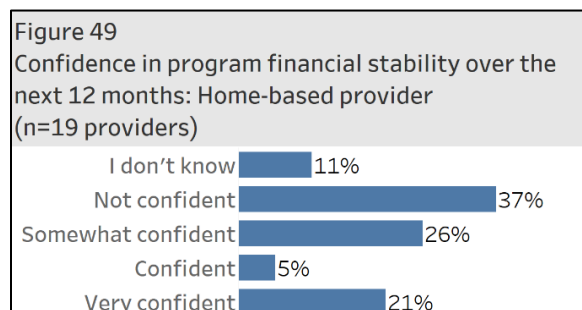
When asked to rate confidence level on a four-point scale from “Not Confident” to “Very Confident”, a little over a fifth of responding providers reported not feeling confident in their financial stability over the next year. Slightly less than half (47%) reported being confident or very confident in their financial stability (Figure 46).



For-profit providers reported lower rates of feeling confident in their financial stability compared to non-profit providers (44% and 53% respectively). Additionally, almost a third of for-profit providers (29%) were not confident in their financial stability compared to only 14% of non-profit providers (Figures 47 & 48).

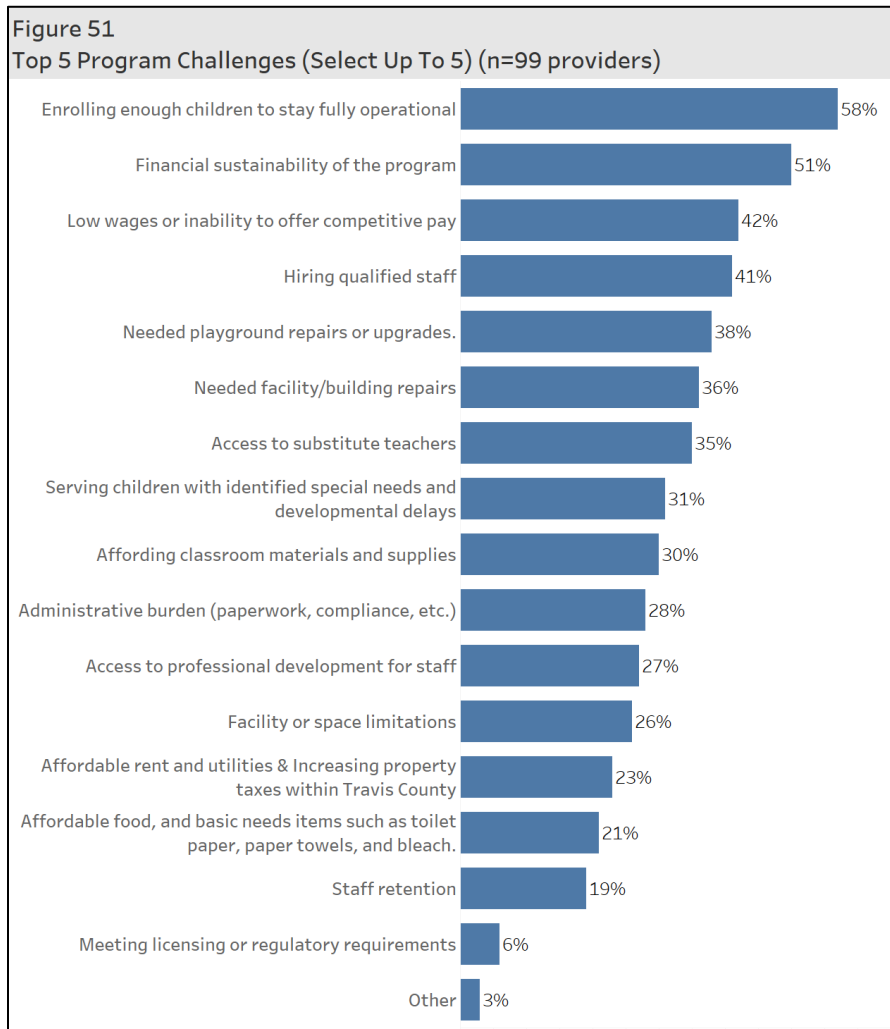


Home-based providers reported being especially uncertain about their financial sustainability. 37% reported not being confident in their financial stability, nearly twice the rate reported by licensed child care centers (19%), highlighting how vulnerable home-based programs are within the broader ECE landscape (Figures 49 & 50).



Top Challenges

To understand how funders and advocates can best support ECE providers, it is important to understand what they perceive as their biggest challenges. Providers were given a list of 16 challenges and asked to select the five they found most significant. The five most selected were: Enrolling enough children to stay fully operational (58%), financial sustainability of the program (51%), low wages or inability to offer competitive pay (42%), hiring qualified staff (41%), and needed playground repairs or upgrades (38%) (Figure 51). These top-selected challenges point to an ECE sector under significant operational and financial strain. As demonstrated in the preceding data, providers are struggling with deeply interconnected systemic issues such as maintaining full enrollment, attracting and retaining qualified staff, and sustaining programs financially. The prominence of low wages and hiring difficulties highlight ongoing workforce instability, while the need for playground and facility repairs suggests that many providers are simultaneously grappling with aging facilities and rising costs of materials and services.



Funding Priorities

In order to align funding with what ECE providers actually need, it is essential to understand how they would prioritize increased government or corporate funding. The survey asked respondents to rate funding priorities from 1 (highest priority) to 11 (lowest priority). Because this question has been included in past United Way's ECE landscape surveys, it offers a valuable view into how provider priorities have shifted over time. Increasing staff salaries has been ranked as the highest priority since 2012. The other top three priorities were all related to staff benefits, education, or training. The fifth priority was related to upgrading facilities.

Relative to the survey in 2016, increasing number of teachers, reducing parent fees, and offering or increasing parent education saw a reduction in prioritization. Offering or increasing parent education was ranked as the lowest priority. On the other hand, improving staff training, upgrading facilities, and increasing the number of non-teaching staff all saw an increase in prioritization.

2025 Funding Priorities

1. Increase staff salaries
2. Improve staff training ↑
3. Offer or increase financial support for teacher education ↑
4. Offer or improve staff benefits
5. Upgrade facilities ↑
6. Increase number of teachers ↓
7. Increase non-teacher staff ↑
8. Serve a higher number of children from low-income families
9. Increase/upgrade program supplies or equipment
10. Reduce parent fees ↓
11. Offer or increase parent education ↓

2016 Funding Priorities

1. Increase staff salaries
2. Increase number of teachers
3. Improve staff training
4. Offer or improve staff benefits
5. Offer/Increase financial support for teacher education
6. Reduce parent fees
7. Offer/Increase parent education
8. Serve a higher number of children from low-income families
9. Increase program supplies and/or equipment
10. Upgrade facilities
11. Increase number of non-teacher staff

**Arrows indicate whether priority level has increased or decreased since 2016.*

Key Take Aways, Lessons Learned, and Next Steps

The Austin/Travis County 2025 Early Care and Education Landscape Survey provides important and much needed insight into the current early care and education system and highlights both the strengths and challenges providers face. The sections that follow highlight key takeaways lessons learned, and next steps.

Key Takeaways

There are several bright spots in the local ECE survey data. Many providers are quality rated, with the majority rated TRS 4-Star or holding national accreditation. A substantial share of teachers and directors also had long tenure and formal education in child development or early care and education. Compensation, while still a challenge across the sector, showed some encouraging signs with average hourly wages for lead teachers reported as \$20 per hour, and more than forty percent of directors earning over \$60,000 annually.

However, these findings also highlight the interconnected challenges that make it difficult to operate stable, high-quality programs. Providers reported difficulty keeping classrooms fully enrolled, retaining existing teachers, recruiting qualified staff, maintaining facilities, and keeping tuition affordable. On average, providers reported operating at just 65% of their licensed capacity, and 72% of desired capacity with approximately 2,562 open slots. Only 16% of providers were fully enrolled, while 84% had open slots in three or more age groups. Over one-fifth of providers were not confident in their financial stability over the next year. Teacher turnover exceeded 40%, driven by low wages, career changes, and performance-related dismissals. Well over half of the providers indicated that open positions remained unfilled largely because applicant pools were small and candidates lacked required qualifications. These staffing challenges often led to significant operational shifts, such as additional workload for current teachers, closing or combining classrooms, or inability to meet quality standards, further straining an already fragile system. At the same time, rising labor and benefit costs, higher costs for food, classroom materials, and facility maintenance, are adding additional challenges. About a third of providers identified the inability to pay competitive wages and benefits as major hiring barriers. Facility upgrades ranked among both the top concerns, and top funding priorities. Families also feel the financial strain which might make affording child care difficult. The average monthly tuition across all ages was \$1,260, with infant tuition exceeding \$1,400. For many families, especially those with infants or multiple children, these costs are unaffordable.

These findings help illuminate where the Austin and Travis County ECE system is most vulnerable, and where targeted support can have the greatest impact. Top funding priorities for providers seem to align closely with top challenges and include increasing staff wages, providing or improving staff benefits, supporting staff education or training, and upgrading facilities.

Lessons Learned and Future Improvements

The survey generated several lessons learned and highlighted clear opportunities for improvement that can inform the next iteration of the ECE landscape survey.

To simplify data collection and outreach, a single survey was administered to both licensed child care centers and home-based providers. While this approach simplified logistics, it resulted in a longer survey and limited the ability to tailor questions to each provider type. Future surveys should be administered separately to reduce burden and improve relevance and data quality.

United Way recognized early on that the survey length could pose a barrier to completion. However, given that more than seven years had passed since the last ECE landscape survey, and the post-COVID environment has fundamentally reshaped the sector, a comprehensive approach was prioritized. Establishing a strong baseline was especially important ahead of the disbursement of the Raising Travis County funds, ensuring that future investments are grounded in an accurate understanding of current conditions. Looking ahead, future surveys should be substantially shorter. A core set of questions should be asked annually to track key indicators, while rotating additional topics every few years. This approach maintains consistent data collection without overburdening providers.

After the survey launched, United Way staff quickly realized that even with substantial incentives, collecting responses was more difficult than in previous years. While the exact reasons are unclear, providers appear to have less available time and are navigating more complex challenges, making a lengthy survey especially hard to complete. Response rates were lower than desired for home-based and corporate providers. Sixty home-based providers did not have email addresses, and many could not be reached by phone, further limiting opportunities to connect. United Way is still building deeper engagement with home-based providers and stronger relationships are expected as this work expands. Corporate providers also proved difficult to reach; although several responded, two large companies with several sites could not be contacted despite multiple attempts. Strengthening relationships with these organizations remains a priority for the coming year.

Timing of the survey may also have been a challenge. The survey initially was supposed to launch mid-September, but delays in securing funding and the need for a more thorough survey-development process - including time to gather provider feedback—pushed the release to late October. As a result, the survey period quickly ran into the holiday season just a few weeks after launch. There is rarely a perfect time for ECE providers, but identifying a more manageable time for providers will be crucial for future success.

Incentives played a key role in encouraging participation. Because we were not sure how many providers would complete the survey, we did not want to promise an amount we could not fully fund, while still ensuring everyone received something for their time. In hindsight, participation may have been stronger with fewer but higher-value gift cards or with funding secured upfront to guarantee a higher incentive for all respondents.

Next steps

This report is United Way's first step in sharing an initial overview of the data with providers, funders, coalition members, and the broader community. The plan is to build on this work with a series of smaller, topic-specific reports and white papers (for example, on quality, non-traditional-hours care, wages and benefits), as well as dashboards on the Early Childhood Data Hub. In the months ahead, United Way will work with providers, coalition partners, funders, and community members to interpret the findings together and identify priority areas where deeper analysis is needed.

THANK YOU TO OUR FUNDERS

for their partnership and generous support
of this survey and report



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